LIBBY — RECOMMENDATIONS

Recommendations — pdac-recurrent-cldn18-2-kras-g12r-tp53-y220c-p25m

Ranked therapeutic options with likelihood, toxicity, mechanism risk, and key references

Generated 2026-08-12

Recommendations — pdac-recurrent-cldn18-2-kras-g12r-tp53-y22

Profile snapshot

- **Primary site:** pancreas
- **Histology:** ductal adenocarcinoma (PDAC; per user framing, consistent with the supplied pathology narrative; the histology line of the path report was not quoted at intake)
- **Stage:** Recurrent PDAC. New pancreatic body mass (July 2026) with likely vascular invasion, roughly 4 months after completing adjuvant FOLFIRINOX (March 2026). Prior course: neoadjuvant mFOLFIRINOX x6 (2025), then R0 resection (December 2025) with 0/34 nodes and CAP tumor regression grade 2 (partial response: evident regression, but more than single cells or rare small groups of residual cancer). The recurrence is radiographic only so far; it has not been biopsied or re-sequenced.
- **ECOG:** 0
- **Age band:** 50-59
- **Sex:** unknown
- **Biomarkers:** CLDN18.2 positive, moderate-to-strong: 2+/3+ membranous staining in 60% of tumor cells (resected primary, December 2025 specimen); CLDN18.2 (recurrent lesion) not assessed; the recurrence has not been biopsied. The 60% 2+/3+ result is from the resected primary only. (ihc_pending); KRAS G12R KRAS p.G12R, pathogenic driver; TP53 Y220C TP53 p.Y220C; KMT2C S2816fs KMT2C p.S2816fs, frameshift; Microsatellite instability (MSI) negative (MSS); Tumor mutational burden (TMB) 0.83 mutations/Mb (low); Germline findings (incidental) none detected, per incidental germline reporting on the tumor NGS report (ngs_pending); Somatic profile (recurrent lesion) not assessed; the mutation list (KRAS G12R, TP53 Y220C, KMT2C S2816fs) is from the December 2025 resection specimen (ngs_pending)
- **Efficacy/toxicity weight:** 0.7

37 ranked therapeutic options across 6 targetable features. Biomarker workup steps live in the standalone Target validation paths report.

Recurrence Profile interventions

1. Daraxonrasib (RMC-6236, oral pan-RAS(ON) multi-selective inhibitor) via RASolute 303, the phase 1/2, or the FDA-authorized expanded-access protocol *(conditional on recurrence_profile positive)*

Likelihood of effect: Moderate: the only RCT-grade PDAC evidence containing this allele (RASolute 302, HR 0.40), read across a pooled estimand and a different line context.

Toxicity burden: High (grade >=3 AEs 61.8%, below the 69.6% chemotherapy comparator: rash, diarrhea, nausea, stomatitis)

Counter-productive MoA:Low (Pan-RAS inhibition can select RAS-wild-type escape; allele-specific EGFR rebound described in preprint only.)

Overall:The only option here with randomized survival evidence in PDAC that contains this allele; what is uncertain is the width of the interval, not the position.

Key references: PMID 42223072 · PMID 42090791 · NCT07573215

2. IBI343 (CLDN18.2 antibody-drug conjugate, exatecan payload) via the phase 1 PDAC cohort *(conditional on recurrence_profile positive)*

Likelihood of effect: Moderate-to-low: 22.7% confirmed ORR in expression-selected treated PDAC, but median OS 9.1 months sits inside the 8.8-month chemotherapy class-switch floor.

Toxicity burden: High (grade >=3 TRAEs 52.6% in the gastric cohort: neutropenia 28.4%, leukopenia 25.9%, anemia, thrombocytopenia)

Counter-productive MoA:Low (Roughly 40% of cells stain antigen-negative; bystander exatecan mitigates but does not erase escape.)

Overall:The best-evidenced CLDN18.2 route this patient can actually reach, and its own median survival has not yet cleared the chemotherapy floor it would displace.

Key references: doi:10.1200/JCO.2025.43.16_suppl.4017 · PMID 40670773 · NCT05458219

3. ASP2138 (CLDN18.2 x CD3 2+1 bispecific T-cell engager), phase 1 with a dedicated metastatic PDAC cohort *(conditional on recurrence_profile positive)*

Likelihood of effect: Low-to-unknown in PDAC: no efficacy readout in this disease; the gastric signal is a 12-week unconfirmed DCR of 41.7%, the softest endpoint in the file.

Toxicity burden: Moderate (CRS 43.2% gastric / 23.4% PDAC cohort, largely low-grade; nausea 44.7%; on-target gastric mucosal injury)

Counter-productive MoA:Moderate (Desmoplastic, T-cell-cold PDAC starves a CD3 engager of the effector density its response depends on.)

Overall:The widest-open CLDN18.2 door in the file, ranked on eligibility and site count rather than on evidence, in the tumor type where engagers historically struggle most.

Key references: NCT05365581 · PMID 34433637 · PMID 23900716

4. ASP5834 (intravenous pan-KRAS inhibitor), first-in-human phase 1 *(conditional on recurrence_profile positive)*

Likelihood of effect: Unknown: zero published efficacy for this molecule. Ranked on eligibility fit, as the KRAS route that survives a locally-recurrent staging call.

Toxicity burden: Unknown (first-in-human dose escalation; no published AE table at any dose)

Counter-productive MoA:Low (Pan-KRAS inhibition can select RAS-wild-type escape; the class resistance map is still being drawn.)

Overall:Insurance against the largest access risk in this case: it accepts locally advanced unresectable

disease, which the daraxonrasib expanded-access protocol and RASolute 303 do not.

Key references: NCT07094204 · PMID 42223072

5. Zolbetuximab (anti-CLDN18.2 monoclonal antibody), off-label in PDAC *(conditional on recurrence_profile positive)*

Likelihood of effect: Low: the only randomized CLDN18.2 readout in PDAC was flat at 13.7 vs 13.6 months, and this tumor sits below the label-defining cutoff.

Toxicity burden: High (grade >=3 TEAEs 87% with mFOLFOX6, 72.8% with CAPOX: nausea, vomiting, neutropenia)

Counter-productive MoA:Low (A naked antibody needs uniform surface antigen; roughly 40% of cells stain negative and there is no payload or crossfire.)

Overall:Prescribable off-label today and argued against on both counts: 60% is below the companion-diagnostic cutoff, and the randomized PDAC readout was flat.

Key references: PMID 37068504 · PMID 37524953 · NCT03816163

Also considered — not ranked

Feature-targeting investigational options the board considered but did not rank as live top-tier choices: Unavailable (program discontinued), Consolidated (one product of an approach ranked above), Thin evidence (no peer-reviewed clinical efficacy yet), or Not enrollable (cross-tumor or geographically inaccessible).

JAB-30355 (oral p53 Y220C reactivator), phase 1/2 with US sites — _Thin evidence_

Likelihood of effect: Unknown, and the class points the wrong way: every responder in rezatapopt's phase 1 (n=77) was KRAS wild-type, ORR 30% in that subset against 20% overall.

Toxicity burden: Unknown (no published AE table at any dose; the entire record is one sponsor sentence)

Counter-productive MoA:Moderate (Restored p53 in a KRAS-driven, low-priming tumor produces arrest rather than apoptosis.)

Overall:The only Y220C door this KRAS-co-mutated patient can walk through, and Y220C fails twice here; the rank measures door-openness, not evidence.

Key references: NCT06386146 · PMID 41740031 · PMID 34074758

NTS071 (oral p53 Y220C reactivator), phase 1/2 with US sites — _Consolidated_

Likelihood of effect: Unknown: consolidated under the JAB-30355 row, which carries the class read for this patient.

Toxicity burden: Unknown (no published safety data of any kind)

Counter-productive MoA:Moderate (Same class liability: restored p53 in a KRAS-driven, low-priming tumor arrests rather than kills.)

Overall:The second US Y220C door without a posted KRAS exclusion, kept as redundancy if the first one narrows.

Key references: NCT07060989

KST-6051 (oral pan-KRAS inhibitor), first-in-human phase 1 at US sites — _Thin evidence_

Likelihood of effect: Unknown: no published efficacy. Eligibility as posted asks only for a documented KRAS mutation, which G12R satisfies on its face.

Toxicity burden: Unknown (first-in-human dose escalation; no published AE table)

Counter-productive MoA:Low (Pan-KRAS inhibition can select RAS-wild-type escape; no drug-specific mechanism data exist.)

Overall:A reachable US pan-KRAS door with an unusually simple eligibility line, and it closes permanently once daraxonrasib starts.

Key references: NCT07458347

AN9025 (pan-RAS(ON) inhibitor), phase 1 at three US sites — _Thin evidence_

Likelihood of effect: Unknown: no published efficacy. Its posted criteria fit this history unusually well, including adjuvant therapy with progression inside 6 months as prior therapy.

Toxicity burden: Unknown (first-in-human dose escalation; no published AE table)

Counter-productive MoA:Low (Pan-RAS(ON) inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:The rare pan-RAS(ON) trial with backfill cohorts that accept patients who already progressed on daraxonrasib, which makes it a post-progression asset.

Key references: NCT07252479

JAB-23E73 (oral pan-KRAS inhibitor), phase 1/2 — _Consolidated_

Likelihood of effect: Consolidated under the ranked pan-KRAS approach; no drug-specific efficacy published.

Toxicity burden: Unknown (dose escalation; no published AE table)

Counter-productive MoA:Low (Pan-KRAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:One more recruiting pan-KRAS escalation with no published data; the approach is already ranked at daraxonrasib and ASP5834.

Key references: NCT06973564

BLU-924 / SAR449336 (pan-KRAS inhibitor), phase 1/2 — _Consolidated_

Likelihood of effect: Consolidated under the ranked pan-KRAS approach; no drug-specific efficacy published.

Toxicity burden: Unknown (dose escalation; no published AE table)

Counter-productive MoA:Low (Pan-KRAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:One more recruiting pan-KRAS escalation with no published data; the approach is already ranked at daraxonrasib and ASP5834.

Key references: NCT07629960

BBO-11818 (pan-KRAS inhibitor), phase 1 — _Consolidated_

Likelihood of effect: Consolidated under the ranked pan-KRAS approach; no drug-specific efficacy published.

Toxicity burden: Unknown (dose escalation; no published AE table)

Counter-productive MoA:Low (Pan-KRAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:One more recruiting pan-KRAS escalation with no published data; the approach is already ranked at daraxonrasib and ASP5834.

Key references: NCT06917079

Satri-cel (CT041, autologous CLDN18.2 CAR-T) — _Not enrollable_

Likelihood of effect: Not reachable: the strongest design in the CLDN18.2 class (mPFS 3.25 vs 1.77 months, HR 0.37) is gastric, and the absolute gain is about six weeks.

Toxicity burden: High (grade >=3 TEAEs in 87 of 88 treated patients; CRS 95%; lymphopenia 98%; on-target gastric mucosal injury)

Counter-productive MoA:Moderate (Desmoplastic, T-cell-poor PDAC is the environment CAR-T does worst in; on-target gastric injury adds a second vector.)

Overall:The closest expression match in the file and the one with no door: North American enrollment closed, the approval is dispensed only in mainland China.

Key references: PMID 40460847 · PMID 38830992 · PMID 37689733

[177Lu]Lu-DOTA-SNA040 (CLDN18.2-directed lutetium-177 radioligand therapy), investigator-initiated phase 1 — _Not enrollable_

Likelihood of effect: Unknown clinically, but the biomarker fit is the best in the file: it requires >=2+ in >=20% of cells and this tumor is 2+/3+ in 60%.

Toxicity burden: Unknown (no published clinical safety; radioligand class carries marrow and renal dose constraints)

Counter-productive MoA:Low (Beta crossfire kills antigen-negative neighbours, which is the one CLDN18.2

approach heterogeneity does not disable.)

Overall:The best mechanistic match in the whole dossier and the least reachable: a 12-patient single-center trial in Shanghai with no mechanism for foreign enrollees.

Key references: NCT07595237

Spevatamig (PT886, CLDN18.2 x CD47 bispecific), phase 1/2 at 11 US sites — _Thin evidence_

Likelihood of effect: Unknown: no published efficacy. A straightforward US door in the CLDN18.2 class with tissue assessment built into screening.

Toxicity burden: Unknown (no published AE table; CD47 engagement carries a class anemia signal)

Counter-productive MoA:Low (CD47 blockade recruits macrophages rather than T cells, which sidesteps the T-cell-cold problem but is unproven here.)

Overall:One of the few CLDN18.2 programs recruiting at US sites for previously treated pancreatic carcinoma, with no published data behind it.

Key references: NCT05482893

AZD5863 (CLDN18.2 x CD3 affinity-modulated T-cell engager), phase 1/2 — _Consolidated_

Likelihood of effect: Consolidated under the ranked CD3-engager approach; preclinical work reports potent activity with limited cytokine release, and no clinical efficacy is published.

Toxicity burden: Unknown clinically (the affinity-modulated design is built to lower cytokine release; no human AE table)

Counter-productive MoA:Moderate (Same T-cell-density dependency that handicaps every CD3 engager in desmoplastic PDAC.)

Overall:A second CLDN18.2 x CD3 engager whose affinity modulation targets the CRS problem; consolidated under the ASP2138 rank.

Key references: NCT06005493 · PMID 40759445

Evidence in detail

Daraxonrasib (RMC-6236, oral pan-RAS(ON) multi-selective inhibitor)

O'Reilly et al. *N Engl J Med* 2026

- **Disease context:Previously treated metastatic PDAC (91.8% RAS G12-mutant)** (2L) — Previously treated mPDAC; dual primary endpoints in the RAS G12 subpopulation, which contains this patient's G12R. Overall population also took G13/Q61 and RAS-WT.
- **n:** 500
- **Toxicities:any AE** (grade any): 100% (n=248); **any AE** (grade >=3): 61.8% (n=248); **TRAE leading to discontinuation** (grade any): 1.2% (n=248)
- **Efficacy:OS** 13.2 months; **OS** 13.2 months; **HR_OS** 0.4; **PFS** 7.3 months; **HR_PFS** 0.45; **HR_PFS** 0.49;

AE_rate 61.8%; discontinuation_rate 1.2%

Wolpin et al. *N Engl J Med* 2026

- **Disease context:Previously treated advanced RAS-mutated PDAC (phase 1/2 solid-tumor study, PDAC subset)** (2L+) — 168 previously treated RAS-mutant PDAC patients dosed at <=300 mg; key efficacy subgroup is 26 patients with RAS G12 tumors on second-line 300 mg. G12R qualifies alongside the other G12 alleles.
- **n:** 168
- **Toxicities:any TRAE** (grade any): 96% (n=168); **any TRAE** (grade >=3): 30% (n=168); **rash** (grade any): (n=168); **diarrhea** (grade any): (n=168); **nausea** (grade any): (n=168); **stomatitis/mucositis** (grade any): (n=168); **vomiting** (grade any): (n=168); **fatigue** (grade any): (n=168)
- **Efficacy:ORR** 35% (95% CI 17-56); **DoR** 8.2 months; **PFS** 8.5 months; **OS** 13.1 months; **ORR** 29% (95% CI 15-46); **DoR** 8.2 months (95% CI 3.8-8.8); **PFS** 8.1 months; **OS** 15.6 months; **TRAE_rate** 96%; **TRAE_rate** 30%

IBI343 (CLDN18.2 antibody-drug conjugate, exatecan payload)

Liu et al. *Nat Med* 2025

- **Disease context:Advanced gastric/GEJ adenocarcinoma, CLDN18.2-positive (phase 1)** (2L+) — 127 dosed (19 escalation, 108 expansion); headline efficacy in CLDN18.2-high (2+/3+ >=75%) at 6 mg/kg.
- **n:** 127
- **Toxicities:any TRAE** (grade >=3): 52.6% (n=116); **neutrophil count decreased** (grade >=3): 28.4% (n=116); **white blood cell count decreased** (grade >=3): 25.9% (n=116); **anemia** (grade >=3): 16.4% (n=116); **ILD/pneumonitis** (grade any): 0% (n=127); **treatment-related death** (grade 5): 0% (n=116)
- **Efficacy:ORR** 29% (95% CI 14.2-48.0); **PFS** 5.5 months (95% CI 4.1-7.0); **TRAE_rate** 52.6%; **discontinuation_rate** 0.9%

ASP5834 (intravenous pan-KRAS inhibitor), first-in-human phase 1

O'Reilly et al. *N Engl J Med* 2026

- **Disease context:Previously treated metastatic PDAC (91.8% RAS G12-mutant)** (2L) — Previously treated mPDAC; dual primary endpoints in the RAS G12 subpopulation, which contains this patient's G12R. Overall population also took G13/Q61 and RAS-WT.
- **n:** 500
- **Toxicities:any AE** (grade any): 100% (n=248); **any AE** (grade >=3): 61.8% (n=248); **TRAE leading to discontinuation** (grade any): 1.2% (n=248)
- **Efficacy:OS** 13.2 months; **OS** 13.2 months; **HR_OS** 0.4; **PFS** 7.3 months; **HR_PFS** 0.45; **HR_PFS** 0.49; **AE_rate** 61.8%; **discontinuation_rate** 1.2%

Zolbetuximab (anti-CLDN18.2 monoclonal antibody), off-label in PDAC

Shitara et al. *Lancet* 2023

- **Disease context:CLDN18.2-positive (>=75% 2+/3+), HER2-negative, untreated advanced gastric/GEJ adenocarcinoma** (1L) — 565 randomized across 215 centers; ECOG 0-1.
- **n:** 565
- **Toxicities:any TEAE** (grade >=3): 87% (n=279); **nausea** (grade >=3): (n=279); **vomiting** (grade >=3): (n=279); **decreased appetite** (grade >=3): (n=279); **treatment-related death** (grade 5): 2% (n=279)
- **Efficacy:PFS** 10.61 months (95% CI 8.9-12.48); **HR_PFS** 0.75 (95% CI 0.6-0.94); **HR_OS** 0.75 (95% CI

0.6-0.94); **AE_rate** 87%

Shah et al. *Nat Med* 2023

- **Disease context:CLDN18.2-positive (>=75% 2+/3+), HER2-negative, untreated advanced gastric/GEJ adenocarcinoma** (1L) — 507 randomized 1:1, global.
- **n:** 507
- **Toxicities:any TEAE** (grade >=3): 72.8%
- **Efficacy:PFS** 8.21 months; **HR_PFS** 0.687 (95% CI 0.544-0.866); **OS** 14.39 months; **HR_OS** 0.771 (95% CI 0.615-0.965); **AE_rate** 72.8%

JAB-30355 (oral p53 Y220C reactivator), phase 1/2 with US sites

Dumbrava et al. *N Engl J Med* 2026

- **Disease context:Locally advanced/metastatic solid tumors with TP53 Y220C (heavily pretreated)** (2L+) — 77 patients across 8 dose levels. Every responder had a KRAS wild-type tumor; ORR 30% in KRAS-WT at >=1150 mg vs 20% overall.
- **n:** 77
- **Toxicities:nausea** (grade any): 58% (n=77); **vomiting** (grade any): 44% (n=77); **blood creatinine increased** (grade any): 39% (n=77); **fatigue** (grade any): 39% (n=77); **anemia** (grade any): 36% (n=77); **anemia** (grade >=3): 16% (n=77); **TRAE leading to discontinuation** (grade any): 3% (n=77)
- **Efficacy:ORR** 20%; **ORR** 30%; **AE_rate** 99%; **TRAE_rate** 87%; **discontinuation_rate** 3%

Satri-cel (CT041, autologous CLDN18.2 CAR-T)

Qi et al. *Lancet* 2025

- **Disease context:CLDN18.2-positive (2+ intensity in >=40% of cells) advanced gastric/GEJ adenocarcinoma, >=2 prior lines** (2L+) — 156 randomized 2:1 in China; 69% of the satri-cel group had peritoneal metastasis. The >=2+ in >=40% eligibility rule is the one this patient's 60% at 2+/3+ clears.
- **n:** 156
- **Toxicities:any TEAE** (grade >=3): 99% (n=88); **lymphocyte count decreased** (grade >=3): 98% (n=88); **white blood cell count decreased** (grade >=3): 77% (n=88); **neutrophil count decreased** (grade >=3): 66% (n=88); **CRS** (grade any): 95% (n=88)
- **Efficacy:PFS** 3.25 months (95% CI 2.86-4.53); **HR_PFS** 0.37 (95% CI 0.24-0.56); **AE_rate** 99%

Qi et al. *Nat Med* 2024

- **Disease context:CLDN18.2-positive advanced gastrointestinal cancers (gastric-dominant; includes pancreatic patients)** (2L+) — 98 infused across escalation and four expansion cohorts (monotherapy, +anti-PD-1, post-1L sequential, anti-CLDN18.2-mAb-refractory).
- **n:** 98
- **Toxicities:CRS** (grade any): 96.9% (n=98); **gastric mucosal injury** (grade any): 8.2% (n=98); **ICANS** (grade any): 0% (n=98); **treatment-related death** (grade 5): 0% (n=98)
- **Efficacy:ORR** 38.8%; **DCR** 91.8%; **PFS** 4.4 months (95% CI 3.7-6.6); **OS** 8.8 months (95% CI 7.1-10.2); **AE_rate** 96.9%

Qi et al. *J Hematol Oncol* 2023

- **Disease context:CLDN18.2-positive metastatic PDAC after failure of standard therapy** (2L+) — Two patients: case 1 CLDN18.2 2+ in 70%; case 2 3+ in 60% — case 2 is nearly this patient's exact expression profile.

- **n:** 2
- **Toxicities:CRS** (grade 1-2): 100% (n=2)
- **Efficacy:RR** PR; **RR** target-lesion CR (lung metastases)

Germline BRCA / HRD-targeting interventions

1. Olaparib maintenance, gated on the dedicated germline panel that has not been drawn

Likelihood of effect: Low: roughly 5-7% pre-test probability of germline BRCA1/2, a POLO population fit this patient does not currently meet, and PFS-only benefit (HR 0.53).

Toxicity burden: Moderate (grade >=3 AEs 40% vs 23% on placebo: anemia, fatigue, nausea)

Counter-productive MoA:Low (Recurrence four months after platinum-containing adjuvant therapy argues against the platinum sensitivity this strategy assumes.)

Overall:Ranked chiefly to force a blood draw NCCN has asked for in every PDAC patient since 2019, which gates the one FDA-labelled biomarker route in this disease.

Key references: PMID 31157963 · PMID 35834777

Evidence in detail

Olaparib maintenance, gated on the dedicated germline panel that has not been drawn

Golan et al. *N Engl J Med* 2019

- **Disease context:Germline BRCA1/2-mutated metastatic PDAC, non-progressing on >=16 weeks of first-line platinum** (maintenance) — 154 randomized 3:2 of 3315 screened — the screening funnel itself shows how uncommon the gBRCA gate is.
- **n:** 154
- **Toxicities:any AE** (grade >=3): 40% (n=91); **AE leading to discontinuation** (grade any): 5% (n=91)
- **Efficacy:PFS** 7.4 months; **HR_PFS** 0.53 (95% CI 0.35-0.82); **AE_rate** 40%; **discontinuation_rate** 5%

Golan et al. *J Clin Oncol* 2022

- **Disease context:Germline BRCA1/2-mutated metastatic PDAC, non-progressing on >=16 weeks of first-line platinum** (maintenance) — Same 154-patient POLO population; final OS analysis.
- **n:** 154
- **Toxicities:** Long-term tolerability held up; no new safety signals versus the primary analysis.
- **Efficacy:OS** 19.0 months; **HR_OS** 0.83 (95% CI 0.56-1.22); **OS_rate** 33.9%; **TTNT** 0.44 HR (95% CI 0.3-0.66)

Mss Tmb Low interventions

1. Single-agent PD-1 blockade (pembrolizumab, MSI-high / TMB-high tumor-agnostic

routes)

Likelihood of effect: Low: MSS with TMB 0.83 mut/Mb forecloses both tumor-agnostic indications; single-agent checkpoint blockade has no route in this tumor.

Toxicity burden: Low (immune-related AE profile; academic here, since the indication is closed on biomarker)

Counter-productive MoA:Low (No target present; FAP+ CAF-driven exclusion keeps single-agent checkpoint blockade inert in MSS PDAC.)

Overall:Carried so the MSS / TMB-low result is visibly answered: it closes checkpoint monotherapy and routes immune interest to engagers, vaccines, and cell therapy instead.

Key references: NCT02628067 · PMID 24277834

Tp53 Y220C interventions

Also considered — not ranked

Feature-targeting investigational options the board considered but did not rank as live top-tier choices: Unavailable (program discontinued), Consolidated (one product of an approach ranked above), Thin evidence (no peer-reviewed clinical efficacy yet), or Not enrollable (cross-tumor or geographically inaccessible).

Rezatapopt (PC14586, first-in-class p53 Y220C reactivator), PYNNACLE — _Not enrollable_

Likelihood of effect: Not enrollable as posted: ORR 20% overall and 30% in KRAS wild-type at >=1150 mg (n=77), with no responder carrying a KRAS mutation.

Toxicity burden: Moderate (nausea 58%, vomiting 44%, creatinine increase 39%, fatigue 39%, largely low-grade)

Counter-productive MoA:Moderate (Acquired secondary TP53 alterations in cis with Y220C drive on-target resistance in this class.)

Overall:The flagship Y220C program, closed to this patient by its own KRAS-SNV exclusion, with a phase 1 responder pattern that says the same thing.

Key references: PMID 41740031 · NCT04585750 · PMID 41504628

GenSci128 (p53 Y220C reactivator), phase 1 — _Not enrollable_

Likelihood of effect: Unknown: no published data, and no reachable site for a US patient.

Toxicity burden: Unknown (no published safety data)

Counter-productive MoA:Moderate (Shares the class liability of p53 reactivation in a KRAS-driven, low-priming tumor.)

Overall:A third Y220C reactivator in the file with no Western route; surfaced so the class inventory is complete.

Key references: NCT06908434

LG00313112 (oral p53 Y220C reactivator), phase 1/2 — _Not enrollable_

Likelihood of effect: Unknown: not yet recruiting, no published data.

Toxicity burden: Unknown (no published safety data)

Counter-productive MoA:Moderate (Shares the class liability of p53 reactivation in a KRAS-driven, low-priming tumor.)

Overall:Not yet recruiting; a registry alert rather than an option.

Key references: NCT07752875

Evidence in detail

Rezatapopt (PC14586, first-in-class p53 Y220C reactivator), PYNNACLE

Dumbrava et al. *N Engl J Med* 2026

- **Disease context:Locally advanced/metastatic solid tumors with TP53 Y220C (heavily pretreated)** (2L+) — 77 patients across 8 dose levels. Every responder had a KRAS wild-type tumor; ORR 30% in KRAS-WT at >=1150 mg vs 20% overall.
- **n:** 77
- **Toxicities:nausea** (grade any): 58% (n=77); **vomiting** (grade any): 44% (n=77); **blood creatinine increased** (grade any): 39% (n=77); **fatigue** (grade any): 39% (n=77); **anemia** (grade any): 36% (n=77); **anemia** (grade >=3): 16% (n=77); **TRAE leading to discontinuation** (grade any): 3% (n=77)
- **Efficacy:ORR** 20%; **ORR** 30%; **AE_rate** 99%; **TRAE_rate** 87%; **discontinuation_rate** 3%

KRAS G12R-targeting interventions

Also considered — not ranked

Feature-targeting investigational options the board considered but did not rank as live top-tier choices: Unavailable (program discontinued), Consolidated (one product of an approach ranked above), Thin evidence (no peer-reviewed clinical efficacy yet), or Not enrollable (cross-tumor or geographically inaccessible).

PF-07934040 (pan-KRAS inhibitor), phase 1 — _Not enrollable_

Likelihood of effect: Not enrollable for this patient (active but not recruiting); no published efficacy.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (Pan-RAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:A pan-KRAS program that matches the allele and has no open door for this patient.

Key references: NCT06447662

YL-17231 (pan-RAS inhibitor), phase 1 — _Not enrollable_

Likelihood of effect: Not enrollable for this patient (recruiting in China only); no published efficacy.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (Pan-RAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:A pan-KRAS program that matches the allele and has no open door for this patient.

Key references: NCT06096974

JYP0015 / ERAS-0015 (pan-RAS(ON) molecular glue), phase 2 — _Not enrollable_

Likelihood of effect: Not enrollable for this patient (graded not-yet-accessible by the access screen); no published efficacy.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (Pan-RAS inhibition can select RAS-wild-type escape; no drug-specific data exist.)

Overall:A pan-KRAS program that matches the allele and has no open door for this patient.

Key references: NCT06895031

mKRASvax + balstilimab + botensilimab (mutant-KRAS peptide vaccine with dual checkpoint blockade), single US site — _Not enrollable_

Likelihood of effect: Low for bulk disease: AMPLIFY-201's HR 0.12 is a T-cell-threshold split inside a single-arm minimal-residual-disease trial with a different vaccine.

Toxicity burden: Unknown for this construct (dual checkpoint blockade brings immune-related toxicity; no organ-function labs exist)

Counter-productive MoA:Moderate (Vaccines rarely shrink bulk desmoplastic tumors; the supporting data come from a minimal-residual-disease setting.)

Overall:G12R is in the peptide pool and the mandatory ECOG 0 fits, but a radiographic mass with vascular invasion is not minimal residual disease.

Key references: NCT06411691 · PMID 40790272

ELI-002 7P (seven-peptide mutant-KRAS amphiphile vaccine), AMPLIFY-7P and the MSK combination study — _Not enrollable_

Likelihood of effect: Not enrollable: AMPLIFY-7P screens out patients whose imaging shows recurrence, which is this patient's presentation.

Toxicity burden: Low (no dose-limiting toxicities across 25 patients in the phase 1; final abstract carries no AE table)

Counter-productive MoA:Low (Minimal-residual-disease biology; no counter-productive vector, but no bulk-disease activity either.)

Overall:The vaccine platform with real G12R immunogenicity data in PDAC patients, and an entry criterion this recurrence fails on sight.

Key references: NCT05726864 · NCT07671339 · PMID 40790272

Pipeline context — not currently enrollable

Other agents in this target class drawn from the case dossier. The patient cannot currently enroll in these trials, but they are included for context on the broader modality landscape.

Intervention	Modality	Phase	Enrolling indication	Recruitment	Trial
zoldonrasib (RMC-9805, KRAS G12D(ON)-selective)	small_molecule	3	Metastatic PDAC, first-line, KRAS G12D-mutant	recruiting	NCT07621718
daratumumab + TG01 + nivolumab	vaccine	2	PDAC after one failed treatment (and NSCLC post-anti-PD-1) with KRAS codon 12/13 mutations	active not recruiting	NCT06015724
daraxonrasib (RMC-6236, oral RAS(ON) multi-selective inhibitor)	small_molecule	1/2	Previously treated advanced RAS-mutated PDAC (phase 1/2 RMC-6236-001; trial still recruiting)	recruiting	NCT05379985
ANOC-001 / ANOC-002 / ANOC-003 (mutant-KRAS TCR-T)	other	1/2	Metastatic/locally advanced PDAC (Anocca TCR-T master protocol)	recruiting	NCT07145450
ELI-002 2P (mKRAS amphiphile vaccine)	vaccine	1	PDAC (20) and CRC (5) with minimal residual mKRAS disease after locoregional treatment (AMPLIFY-201, ELI-002 2P)	completed	NCT04853017
NT-112 / AZD0240 (KRAS G12D-directed TCR-T)	other	1	Unresectable/metastatic solid tumors incl. PDAC with KRAS G12D	recruiting	NCT06218914
TCR1188-ABC (KRAS G12V TCR-T)	other	1	KRAS G12V-mutated pancreatic, cholangiocarcinoma, CRC, NSCLC (UPenn)	recruiting	NCT07594067

Evidence in detail

mKRASvax + balstilimab + botensilimab (mutant-KRAS peptide vaccine with dual checkpoint blockade), single US site

Wainberg et al. *Nat Med* 2025

- **Disease context:PDAC (n=20) and CRC (n=5) in minimal-residual-disease state after locoregional therapy** (adj) — MRD-positive (ctDNA/serum biomarker) after surgery and chemotherapy, radiographically recurrence-free. KRAS G12D or G12R tumors; the vaccine's peptide pool covers this patient's allele exactly.

- **n:** 25
- **Toxicities:dose-limiting toxicity** (grade any): 0% (n=25)
- **Efficacy:HR_RFS** 0.12; **HR_OS** 0.23; **biomarker** 71%; **biomarker** 67%

ELI-002 7P (seven-peptide mutant-KRAS amphiphile vaccine), AMPLIFY-7P and the MSK combination study

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Cldn18 2 Expression interventions

Also considered — not ranked

Feature-targeting investigational options the board considered but did not rank as live top-tier choices: Unavailable (program discontinued), Consolidated (one product of an approach ranked above), Thin evidence (no peer-reviewed clinical efficacy yet), or Not enrollable (cross-tumor or geographically inaccessible).

AZD0901 / CMG901 (CLDN18.2 ADC, MMAE payload), phase 2 with a PDAC cohort — _Not enrollable_

Likelihood of effect: Not enrollable as posted: the PDAC cohort is first-line and requires progression at least 6 months after the last perioperative dose; this recurrence came at about 4 months.

Toxicity burden: High (grade >=3 TEAEs 68% in gastric: neutropenia 21%, anemia 14%, vomiting 10%)

Counter-productive MoA:Low (MMAE payload without meaningful bystander reach; antigen-negative cells are not covered.)

Overall:A broadly recruiting CLDN18.2 ADC whose PDAC cohort excludes this patient by two months of arithmetic.

Key references: PMID 39788133 · NCT06219941

FG-M108 (ADCC-enhanced anti-CLDN18.2 monoclonal antibody) + gemcitabine/nab-paclitaxel, phase 3 — _Not enrollable_

Likelihood of effect: Unknown for this patient: the supporting phase 1b ORR of 32.4% (n=39) sits on a chemotherapy backbone that cannot isolate the antibody's contribution.

Toxicity burden: Moderate (anemia 56.2%, nausea 56.4%, vomiting 48.7% any-grade on a gemcitabine/nab-paclitaxel backbone)

Counter-productive MoA:Low (Naked antibody depends on uniform antigen; ADCC enhancement does not reach antigen-negative cells.)

Overall:The one CLDN18.2 phase 3 whose cutoff this tumor actually clears, posted at a single mainland-China site and not yet recruiting.

Key references: doi:10.1200/JCO.2025.43.4_suppl.729 · NCT07383922

ASP546C (CLDN18.2 ADC), phase 1/2 — _Thin evidence_

Likelihood of effect: Unknown: no published efficacy or safety data.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (ADC class; antigen-negative cells depend on whatever bystander reach the undisclosed payload has.)

Overall:A recruiting CLDN18.2 ADC with nothing published; a screening call rather than a considered option.

Key references: NCT07488676

QLS31905 / ANO31905 (CLDN18.2 x CD3 bispecific) + gemcitabine/nab-paclitaxel, phase 3 and phase 1/2 — _Not enrollable_

Likelihood of effect: Not enrollable: recruiting in China, and the phase 3 pairs the engager with a chemotherapy backbone this patient has functionally failed.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Moderate (Same T-cell-density dependency as the class, on a chemotherapy backbone this tumor already progressed through.)

Overall:A CLDN18.2 engager that reached phase 3 in PDAC, recruiting in China only.

Key references: NCT07079228 · NCT07444541

Givastomig (TJ033721/ABL111, CLDN18.2 x 4-1BB bispecific), phase 1 — _Not enrollable_

Likelihood of effect: Cross-tumor only: ORR 16% in CLDN18.2-positive gastroesophageal cancer at >=5 mg/kg (n=75), with no pancreatic arm.

Toxicity burden: Low (no dose-limiting toxicities through 18 mg/kg; MTD not reached; nausea and anemia the common treatment-related events)

Counter-productive MoA:Moderate (4-1BB costimulation needs T cells already present; PDAC is where that assumption fails.)

Overall:The best-tolerated agent in the CLDN18.2 file, with gastroesophageal-only efficacy and no pancreatic cohort.

Key references: PMID 40586719 · NCT04900818

XNW27011 (CLDN18.2 ADC), phase 1/2 — _Not enrollable_

Likelihood of effect: Not enrollable (active but not recruiting, and graded not-yet-accessible); no case-relevant efficacy published.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (ADC class; antigen-negative cells depend on the payload's bystander reach.)

Overall:A CLDN18.2 ADC with no open door for this patient.

Key references: NCT06792435

AZD4360 (CLDN18.2 ADC), phase 1/2 — _Not enrollable_

Likelihood of effect: Not enrollable (active but not recruiting); no case-relevant efficacy published.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (ADC class; antigen-negative cells depend on the payload's bystander reach.)

Overall:A CLDN18.2 ADC with no open door for this patient.

Key references: NCT06921928

TORL-2-307-ADC (CLDN18.2 ADC), phase 1 — _Not enrollable_

Likelihood of effect: Not enrollable (active but not recruiting); no case-relevant efficacy published.

Toxicity burden: Unknown (no published AE table)

Counter-productive MoA:Low (ADC class; antigen-negative cells depend on the payload's bystander reach.)

Overall:A CLDN18.2 ADC with no open door for this patient.

Key references: NCT05156866

LB1908 (CLDN18.2 CAR-T), phase 1 — _Not enrollable_

Likelihood of effect: Not enrollable: active but not recruiting; no clinical efficacy published in pancreatic cancer.

Toxicity burden: Unknown (cell-therapy class: CRS and lymphodepletion cytopenias; no published grade profile)

Counter-productive MoA:Moderate (Cell therapy in desmoplastic, T-cell-poor PDAC faces the trafficking barrier that dominates this disease.)

Overall:A CLDN18.2 cell-therapy program that has stopped enrolling; carried so the cellular sweep is complete.

Key references: NCT05539430

TAC01-CLDN18.2 (T-cell antigen coupler engineered T cells), phase 1/2 — _Not enrollable_

Likelihood of effect: Not enrollable: active but not recruiting; no clinical efficacy published in pancreatic cancer.

Toxicity burden: Unknown (cell-therapy class: CRS and lymphodepletion cytopenias; no published grade profile)

Counter-productive MoA:Moderate (Cell therapy in desmoplastic, T-cell-poor PDAC faces the trafficking barrier that dominates this disease.)

Overall:A CLDN18.2 cell-therapy program that has stopped enrolling; carried so the cellular sweep is complete.

Key references: NCT05862324 · PMID 39404622

EO-3021 / SYSA1801 (CLDN18.2 ADC, MMAE payload), phase 1 — _Unavailable_

Likelihood of effect: Not available: the program was terminated with no successor study posted.

Toxicity burden: Not assessable (program terminated; no usable published safety readout)

Counter-productive MoA:Low (ADC class; no mechanism-level concern beyond the usual antigen-negative gap.)

Overall:A discontinued CLDN18.2 ADC, carried so its absence reads as a program decision rather than an oversight.

Key references: NCT05980416

Evidence in detail

AZD0901 / CMG901 (CLDN18.2 ADC, MMAE payload), phase 2 with a PDAC cohort

Ruan et al. *Lancet Oncol* 2025

- **Disease context:Advanced gastric/GEJ adenocarcinoma, CLDN18.2-positive (first-in-human phase 1)** (2L+) — 27 escalation + 107 gastric/GEJ expansion in China; refractory to or lacking standard therapy; pancreatic dose-expansion data held for a separate publication.
- **n:** 134
- **Toxicities:any TEAE** (grade >=3): 68% (n=107); **neutrophil count decreased** (grade 3): 21% (n=107); **anemia** (grade 3): 14% (n=107); **vomiting** (grade 3): 10% (n=107); **pancreatitis (DLT)** (grade 3): (n=27); **treatment-related death** (grade 5): (n=107)
- **Efficacy:ORR** 28% (95% CI 20-38); **ORR** 29% (95% CI 21-39); **AE_rate** 68%

Givastomig (TJ033721/ABL111, CLDN18.2 x 4-1BB bispecific), phase 1

Ku et al. *Clin Cancer Res* 2025

- **Disease context:CLDN18.2-positive advanced solid tumors; efficacy population gastroesophageal cancer** (2L+) — 75 enrolled; CLDN18.2-positive defined as >=1+ on >=1% of cells — responders spanned 11% to 100% expression, i.e. activity well below this patient's 60%.
- **n:** 75
- **Toxicities:dose-limiting toxicity** (grade any): 0% (n=36); **nausea** (grade any): (n=75); **anemia** (grade any): (n=75); **fatigue** (grade any): (n=75)
- **Efficacy:ORR** 16%